

Senate Bill No. 5—Select Committee
on Health and Wellness

CHAPTER.....

AN ACT relating to health care; creating the Statewide Health Care Access and Recruitment Program Account; providing for certain transfers of money out of the Account; requiring a biennial assessment of the health care needs of this State; establishing the Statewide Health Care Access and Recruitment Grant Program to award grants of money to fund certain projects to address shortages of providers of health care or clinical services or expertise identified by the assessment; requiring a grantee to enter into a funding agreement with the Nevada Health Authority; providing for certain oversight of projects funded through the Program; authorizing the Authority to take certain actions in response to certain changes to a funded project or if a grantee fails to comply with a funding agreement or applicable law; prescribing certain requirements to expedite the credentialing and privileging of providers of health care; revising the circumstances under which a registered pharmacist may possess and administer a dangerous drug for the purposes of administering an immunization; requiring the prioritization of certain applications for licensure as a physician or osteopathic physician; requiring certain reports of the Board of Medical Examiners and the State Board of Osteopathic Medicine to include certain information; making appropriations; and providing other matters properly relating thereto.

Legislative Counsel's Digest:

Existing law provides for various programs for the improvement of public health and health care in this State. (NRS 433.702-433.744, 439.4921-439.5297, 439.600-439.690, 439A.111-439A.185, 439A.200-439A.290, 442.710-442.745, 457.230-457.280, 458.025, 458A.090, 458A.100, 458A.110) **Section 10** of this bill establishes the Statewide Health Care Access and Recruitment Grant Program, which is a competitive grant program managed by the Nevada Health Authority to support projects to address critical shortages of providers of health care or clinical services or expertise in this State. **Sections 3-7** of this bill define certain terms, and **section 2** of this bill establishes the applicability of those definitions. **Section 8** of this bill creates the Statewide Health Care Access and Recruitment Program Account to hold money to fund the Program. **Section 8** authorizes the Director of the Authority, with the approval of the Interim Finance Committee, to transfer money from the Account to another account for the purpose of obtaining additional federal financial participation under Medicaid. **Section 25** of this bill authorizes the Interim Finance Committee to issue such approval during a legislative session.

Section 9 of this bill requires the Authority to conduct a biennial assessment of the health care needs of this State, which must identify: (1) the total number of



providers of health care in this State who practice different professions and specialties and; (2) the types of clinical services and expertise and the geographic areas experiencing critical shortages of providers of health care and clinical services or expertise. **Section 27** of this bill provides that such an assessment is not a regulation and is therefore not subject to notice and comment rulemaking. **Section 38** of this bill requires the Authority to utilize existing assessments in lieu of conducting the first scheduled assessment on or before July 1, 2026.

Section 10 authorizes certain entities involved in the provision of health care, governmental entities and certain nonprofit organizations to apply for a grant of money from the Account to support a project to address such critical shortages. **Section 10** imposes certain additional requirements for a project to be eligible to receive such funding, including a requirement that certain applicants secure matching financial or in-kind contributions valued at an amount at least equal to the amount of the grant being sought. **Section 11** of this bill prescribes the required contents of an application for such a grant. **Sections 12 and 13** of this bill prescribe the process for the Authority to review applications and, with the approval of the Interim Finance Committee, award grants. **Section 25** authorizes the Interim Finance Committee to issue such approval during a legislative session.

After a grant is awarded, **section 14** of this bill requires the Authority to enter into a funding agreement with the grantee that outlines the terms and conditions of the grant and the responsibilities of the grantee. **Section 15** of this bill requires a grantee to notify and submit a revised plan to the Authority if: (1) the grantee significantly modifies a funded project; (2) the amount of money available for a funded project changes; (3) a funded project becomes inactive for at least 12 months; or (4) a funded project is terminated. **Section 15** authorizes the Authority to take certain actions in response to such notice. **Section 16** of this bill authorizes the Authority and the Office of Finance to provide certain oversight of a funded project. **Section 17** of this bill authorizes the Authority to suspend or terminate a grant or take certain other actions if it determines that the grantee fails to comply with a funding agreement. **Section 17** also requires the Authority to suspend or terminate a grant if it determines that the grantee has violated applicable law, misused funds or submitted fraudulent information to the Authority. Upon the completion of a funded project and again 10 years after the completion of a funded project, **section 18** of this bill requires the grantee to submit a report to the Authority concerning the project. **Section 26** of this bill includes administering the Program within the general duties of the Authority.

Existing law provides for the licensure and regulation of: (1) physicians, physician assistants, anesthesiologist assistants, genetic counselors, perfusionists and practitioners of respiratory care by the Board of Medical Examiners; and (2) osteopathic physicians, physician assistants and anesthesiologist assistants by the State Board of Osteopathic Medicine. (Chapters 630 and 633 of NRS) **Sections 29 and 33** of this bill require the Board of Medical Examiners and the State Board of Osteopathic Medicine, respectively, to establish by regulation a procedure for prioritizing applications for licensure as a physician or osteopathic physician of applicants who plan to: (1) serve underserved geographic areas or populations in this State as identified by the assessment conducted pursuant to **section 9**; or (2) practice a specialty for which there is a shortage in this State as identified by the assessment conducted pursuant to **section 9**. **Sections 30 and 34** of this bill require those Boards to establish electronic systems to expedite the verification of credentials of providers of health care for the purposes of inclusion in insurance networks and serving on the staff of hospitals. **Sections 31 and 35** of this bill require certain reports submitted by those Boards to the Governor and Legislature to include information relating to the efficiency of the process for licensing



physicians or osteopathic physicians, as applicable. **Section 37** of this bill appropriates money to those Boards to carry out the provisions of **sections 29-31 and 33-35**.

Section 19 of this bill requires a hospital to process at least 95 percent of complete requests from providers of health care for privileges to perform services at the hospital not later than 60 days after receiving all of the information necessary to complete such a request. **Sections 20-24** of this bill make conforming changes to indicate the applicability of, and provide for the administration of, the requirements of **section 19** in the same manner as other requirements governing hospitals, except that **sections 19 and 22-24** provide that a hospital is not subject to penalties for failure to comply with **section 19**.

Existing law authorizes a registered pharmacist to possess and administer a dangerous drug in accordance with the applicable regulations of the State Board of Pharmacy if the pharmacist: (1) is trained in and certified to carry out standards and practices for immunization programs; (2) is authorized to administer immunizations pursuant to a written protocol from a physician; and (3) administers immunizations in compliance with certain standards recommended and approved by the Advisory Committee on Immunization Practices of the Centers for Disease Control and Prevention. (NRS 454.213) **Section 24.5** of this bill revises the third requirement by instead providing that the registered pharmacist must administer immunizations in compliance with certain federal standards and recommendations in effect on January 1, 2025, and any modifications to those standards and recommendations or additional standards and recommendations as the State Board of Health may prescribe by regulation.

EXPLANATION – Matter in **bolded italics** is new; matter between brackets ~~omitted material~~ is material to be omitted.

THE PEOPLE OF THE STATE OF NEVADA, REPRESENTED IN
SENATE AND ASSEMBLY, DO ENACT AS FOLLOWS:

Section 1. Chapter 439A of NRS is hereby amended by adding thereto the provisions set forth as sections 2 to 18, inclusive, of this act.

Sec. 2. *As used in sections 2 to 18, inclusive, of this act, unless the context otherwise requires, the words and terms defined in sections 3 to 7, inclusive, of this act have the meanings ascribed to them in those sections.*

Sec. 3. *“Account” means the Statewide Health Care Access and Recruitment Program Account created by section 8 of this act.*

Sec. 4. *“Certified area of need” means a critical shortage of providers of health care who practice a health care profession or specialty, a critical shortage of providers of health care in a geographic area of this State or a critical shortage of clinical services or expertise in this State or a geographic area of this State, as identified by the assessment conducted pursuant to section 9 of this act.*



Sec. 5. *“Funding agreement” means a funding agreement between the Authority and a grantee entered into pursuant to section 14 of this act.*

Sec. 6. *“Medical facility” has the meaning ascribed to it in NRS 449.0151.*

Sec. 7. *“Provider of health care” has the meaning ascribed to it in NRS 629.031.*

Sec. 8. 1. *There is hereby created in the State General Fund the Statewide Health Care Access and Recruitment Program Account. The Authority shall administer the Account.*

2. Any interest earned on money in the Account, after deducting any applicable charges, must be credited to the Account. Money that remains in the Account at the end of a fiscal year does not revert to the State General Fund, and the balance in the Account must be carried forward to the next fiscal year.

3. Except as otherwise provided in subsection 4, money in the Account must be used to carry out the provisions of sections 2 to 18, inclusive, of this act. The Authority may use not more than 3.5 percent of the money in the Account to pay the administrative costs necessary to carry out those provisions.

4. With the approval of the Interim Finance Committee, the Director of the Authority may transfer money from the Account to another account for the purpose of obtaining additional federal financial participation under Medicaid.

5. The Authority may accept gifts, grants and donations to carry out the provisions of sections 2 to 18, inclusive, of this act. The Director of the Authority shall deposit such gifts, grants and donations into the Account.

Sec. 9. 1. *On or before July 1 of each even-numbered year, the Authority shall:*

(a) Conduct a comprehensive assessment of the health care needs in this State; and

(b) Compile a report of the results of the assessment and submit the report to the Governor and the Director of the Legislative Counsel Bureau for transmittal to the Joint Interim Standing Committee on Health and Human Services and the next regular session of the Legislature.

2. The assessment conducted pursuant to paragraph (a) of subsection 1 must consist of:

(a) A quantitative analysis of the health care workforce in this State, including, without limitation:

(1) A determination of the total number of providers of health care in this State and the total number of providers of



health care in this State who practice different professions and different specialties within those professions;

(2) A determination of the total number of providers of health care who practice in different geographic areas of this State and the total number of providers of health care who practice different professions and different specialties within those geographic areas; and

(3) A comparison of the numbers of providers of health care identified pursuant to subparagraphs (1) and (2) with benchmarks established by the Health Resources and Services Administration of the United States Department of Health and Human Services or nationally recognized organizations that prescribe such benchmarks;

(b) A determination of the most critical shortages in the health care workforce of this State, prioritizing:

(1) Essential health care professions and specialties and essential clinical services or expertise currently experiencing shortages; and

(2) Geographic areas of this State that are experiencing the most critical shortages of providers of health care or clinical services or expertise; and

(c) An identification of unmet needs for specific health technology and therapies, including, without limitation, genomic testing, clinical trials, cellular therapies and palliative care.

3. The report compiled pursuant to paragraph (b) of subsection 1 must include, without limitation:

(a) A summary of the assessment conducted pursuant to paragraph (a) of subsection 1, including, without limitation:

(1) An analysis of shortages of providers of health care, shortages of clinical services or expertise and unmet health needs in this State; and

(2) A description of shortages of providers of health care and the shortages of clinical services or expertise by geographic region, including rural and urban areas;

(b) A prioritized list of recommendations for allocating funding pursuant to sections 2 to 18, inclusive, of this act in a manner that addresses the critical shortages and unmet needs identified in the assessment conducted pursuant to paragraph (a) of subsection 1;

(c) Recommendations for legislation and regulatory changes to improve the recruitment and retention of providers of health care; and



(d) An analysis of the effects of projects funded pursuant to sections 2 to 18, inclusive, of this act on the health care workforce and health needs of this State.

4. As used in this section, “palliative care” means a multidisciplinary and patient- and family-centered approach to specialized medical care for a person with a serious illness, which approach focuses on the care of a patient throughout the continuum of an illness and involves addressing the physical, emotional, social and spiritual needs of the patient, as well as facilitating patient autonomy, access to information and choice of care. The term includes, without limitation, discussion of the goals of the patient for treatment and discussion of treatment options appropriate to the patient, including, where appropriate, hospice care and comprehensive management of pain and symptoms.

Sec. 10. 1. The Authority shall, in accordance with sections 2 to 18, inclusive, of this act, establish and administer the Statewide Health Care Access and Recruitment Grant Program as a competitive program to award grants of money from the Account to entities described in subsection 2 seeking to address shortages of providers of health care or clinical services or expertise and difficulties in accessing health care identified in the assessment conducted pursuant to section 9 of this act. The Authority may adopt any regulations necessary to carry out the provisions of sections 2 to 18, inclusive, of this act.

2. The following entities are eligible to apply for a grant from the Account:

(a) Entities that provide health care or coordinate or otherwise facilitate the provision of health care, including, without limitation:

- (1) Medical facilities and community health clinics;
- (2) Group practices of providers of health care;
- (3) Federally-qualified health centers, as defined in 42 U.S.C. § 1396d(l)(2)(B);
- (4) Accredited institutions of higher education that offer programs to train providers of health care, including, without limitation, residency and fellowship programs for providers of health care;
- (5) Entities that provide behavioral health care;
- (6) Facilities for the treatment of alcohol or other substance use disorders, as defined in NRS 449.00455;
- (7) Detoxification facilities or programs certified pursuant to NRS 458.025;



(8) Substance use disorder prevention coalitions certified pursuant to NRS 458.033; and

(9) Oncology organizations, including, without limitation, oncology organizations that:

(I) Offer programs for transplantation and cellular therapy;

(II) Provide or facilitate the provision of clinical trials;

(III) Host fellowships in oncology and oncology subspecialties; or

(IV) Have been designated as a comprehensive cancer center by the National Cancer Institute, or its successor organization;

(b) Governmental entities; and

(c) Nonprofit organizations that provide direct care to patients, training for providers of health care or services aimed at expanding access to health care.

3. To be eligible for a grant from the Account, a proposed project must:

(a) Be designed to increase the number of providers of health care, improve the expertise of providers of health care or improve access to clinical services or innovative treatments.

(b) Directly address one or more certified areas of need.

(c) Include specific, measurable outcomes to demonstrate an increase in the number of providers of health care, improved access to health care and the enhanced capacity of the health care workforce in a manner that addresses the certified areas of need identified in paragraph (b).

(d) Except for projects proposed by entities described in section 71113 of the One Big Beautiful Bill Act of 2025, Public Law No. 119-21, secure from the Federal Government or any other source, or receive from the Federal Government or any other source a commitment to provide, an amount of matching funds and in-kind contributions for which the total value is at least equal to the amount of the grant for which the applicant is applying. Matching funds from the Federal Government meet the requirements of this section only if the applicant demonstrates that the project qualifies for and is likely to receive such federal money. In-kind contributions may consist of, without limitation:

(1) Construction or procurement of machinery or infrastructure;

(2) Recruitment of providers of health care;

(3) The provision of free health care;



(4) Charitable contributions after the completion of the project; and

(5) Unfunded research.

(e) Demonstrate the potential for financial and operational sustainability after the expiration of the grant, including, without limitation, through:

(1) Plans for continued staffing, budget sustainability and continued allocation of resources; and

(2) An impact assessment concerning the potential long-term effects of the project on survival, quality of life and the experience of patients.

(f) Demonstrate that:

(1) All patient care funded by the grant will be provided in this State; or

(2) After making a good faith effort to determine a manner in which to satisfy the requirement set forth in subparagraph (1), the applicant cannot feasibly satisfy that requirement.

4. Money awarded through a grant from the Account must not be used to supplant money previously budgeted for a proposed project.

Sec. 11. An entity described in subsection 2 of section 10 of this act that wishes to receive a grant from the Account to support a project described in subsection 3 of section 10 of this act must apply to the Authority in the form prescribed by the Authority during an open application period established by the Authority. The application must include, without limitation:

1. A comprehensive proposal for the project to be funded by the grant that outlines:

(a) The goals and objectives of the proposed project;

(b) The specific certified areas of need that the proposed project will address; and

(c) Detailed plans to recruit, retain or train providers of health care.

2. A detailed budget that clearly sets forth the projected costs and sources of funding for the proposed project and the manner in which that funding will be used to pay those projected costs. Such sources of funding must include, without limitation, the matching funds and in-kind contributions described in paragraph (d) of subsection 3 of section 10 of this act, where applicable.

3. If applicable, evidence of approval or commitment from the persons and entities that will provide the matching funds or in-kind contributions described in paragraph (d) of subsection 3 of



section 10 of this act or otherwise contribute to or collaborate on the project.

4. Documentation of the qualifications and relevant experience of the applicant, including, without limitation:

(a) A demonstrated ability to produce measurable, long-term improvements in rates of survival, quality of life and the experience of patients; and

(b) The experience and organizational capacity necessary to implement and sustain the project successfully.

5. Identification of the key personnel who will be involved in the project and any relevant partnerships with persons and entities engaged in the provision of health care, educational institutions, governmental entities or other persons and entities.

6. A timeline for implementing the proposed project that includes, without limitation, defined milestones, measurable performance indicators and a plan for reporting progress and evaluating outcomes at regular intervals.

7. A detailed operational plan for the proposed project that:

(a) Outlines staffing, facilities, equipment and other logistical requirements; and

(b) Addresses potential challenges in recruiting and retaining providers of health care.

8. A plan for sustainability that addresses the manner in which the proposed project will continue to provide benefits after the expiration of the grant, including, without limitation, possible sources of funding, partnerships or strategies for reinvestment.

9. Any additional information required by regulation of the Authority for:

(a) Assessing the feasibility, impact and sustainability of projects and the alignment of projects with the objectives set forth in subsection 3 of section 10 of this act; or

(b) Ensuring accountability and preventing the misuse of funds.

Sec. 12. 1. The Authority shall review applications submitted pursuant to section 11 of this act to determine which applicants will be recommended to receive funding pursuant to section 13 of this act.

2. In reviewing applications and determining which applicants to recommend for funding, the Authority shall:

(a) Give first priority for recommendation to projects that will most effectively address unmet needs;

(b) Give secondary priority for recommendation to projects that will provide the greatest benefit in certified areas of need;



(c) Give tertiary priority for recommendation to projects that will be located in:

(1) Areas designated by the Health Resources and Services Administration of the United States Department of Health and Human Services as having a shortage of providers of health care, including, without limitation:

(I) Areas designated as medically underserved areas or health professional shortage areas with high scores;

(II) Areas designated as health professional shortage areas for primary care; and

(III) Areas with populations that have been designated as medically underserved populations; or

(2) Geographic areas where at least 30 percent of the population is enrolled in Medicaid, as determined by the Authority or the Centers for Medicare and Medicaid Services of the United States Department of Health and Human Services;

(d) Give quaternary priority for recommendation to projects that demonstrate a commitment to ensuring that at least 10 percent of patients who receive care through the project are recipients of Medicaid;

(e) To the extent practicable, make recommendations to fund projects in different geographic areas of this State;

(f) Make recommendations to fund projects in a manner that balances the differing health care needs of this State and takes into account other sources of funding, including, without limitation, federal grant programs, that may be available to applicants in order to use state money efficiently and avoid duplicative funding for the same or similar projects; and

(g) To the extent that such entities submit applications for funding that meet the requirements of section 11 of this act, recommend that at least 2 percent of the available funding be awarded to entities described in section 71113 of the One Big Beautiful Bill Act of 2025, Public Law No. 119-21. Money awarded to such an entity must not be expended for any abortion.

3. The Authority shall consider the following criteria when reviewing applications pursuant to subsection 1:

(a) The severity and urgency of the shortage of providers of health care in the certified areas of need targeted by the proposed project;

(b) The potential of the proposed project to produce measurable, long-term improvements in rates of survival, quality of life and the experience of patients, including, without



limitation, by causing high levels of improvement for underserved populations;

(c) The potential of the proposed project to create lasting improvement in accessibility to health care and the availability of providers of health care in the community affected by the proposed project, including, without limitation, by meeting specific metrics for improvements to access to health care, health care outcomes and the capacity of the health care workforce;

(d) The extent of the proposed investment in and establishment of capital infrastructure to address certified areas of need over the long term and support sustainable access to health care, including, without limitation:

(1) The construction of medical facilities and other health care facilities;

(2) The acquisition of medical equipment or equipment related to the advancement of medical research;

(3) The expansion of capabilities for delivering services through precision medicine, early-phase clinical trials and telehealth; and

(4) The development of facilities for residencies or other training of providers of health care;

(e) The level of financial commitment from sources other than the State, including, without limitation, the degree to which the matching funds and in-kind contributions, if required, exceed the amount required by paragraph (d) of subsection 3 of section 10 of this act;

(f) The level of collaboration between the applicant and other entities in the public and private sectors to enhance the overall impact of the project;

(g) The qualifications and organizational capacity of the applicant to effectively implement and sustain the proposed project, including, without limitation, a demonstrated ability to manage similar projects, meet projections concerning outcomes and maintain compliance with the requirements of sections 2 to 18, inclusive, of this act and any regulations adopted pursuant thereto; and

(h) The degree to which the proposed project will strategically use technology or innovative models of delivering health care in a manner that may reduce costs, improve outcomes and expand access to underserved populations.

4. As used in this section, “telehealth” has the meaning ascribed to it in NRS 629.515.



Sec. 13. 1. After reviewing applications pursuant to section 12 of this act, the Authority shall submit to the Interim Finance Committee recommendations for the awarding of grants from the Account for the approval of the Committee.

2. When making recommendations pursuant to subsection 1, the Authority:

(a) Shall ensure that all applicants for grants from the Account meet the requirements of section 10 of this act;

(b) May recommend funding one or more proposed projects, within the limits of money in the Account;

(c) Shall, to the extent that qualified applicants are available, recommend awarding all of the money in the Account for a fiscal year as grants; and

(d) May not recommend awarding a total amount of money during any funding period that exceeds the amount available in the Account for that funding period.

3. The Authority shall maintain a public record of all decisions to recommend the award of funding to an entity that submits an application pursuant to section 11 of this act or to refuse to make such a recommendation.

4. Upon receiving approval of a recommendation from the Interim Finance Committee pursuant to subsection 1, the Authority shall issue a written notice of the approval to the applicant to whom the recommendation pertains. Such written notice must include, without limitation:

(a) The total amount of money being awarded; and

(b) The schedule of disbursements and specific conditions that will be included in the applicable funding agreement.

Sec. 14. 1. Not later than 30 days after the Interim Finance Committee approves a grantee to receive funding pursuant to section 13 of this act, the Authority shall enter into a funding agreement with the grantee. Such a funding agreement must outline the terms and conditions of the grant and the responsibilities of the grantee in a manner that ensures that the grantee satisfies the requirements and objectives specified in subsection 3 of section 10 of this act.

2. A funding agreement entered into pursuant to subsection 1 must:

(a) Provide for the disbursement of the grant in installments that are contingent on the achievement of specific milestones, goals and indicators of performance relating to increases in the number of providers of health care, improved access to health care and improvements to infrastructure;



(b) Require the grantee to submit documentation that the grantee has achieved the milestones, goals and indicators of performance prescribed in the agreement pursuant to paragraph (a) before money may be disbursed, including, without limitation:

(1) Financial reports detailing expenditures of money and allocations of any matching funds;

(2) Operational progress reports that demonstrate measurable achievements in recruiting providers of health care, improving health care infrastructure or expanding the availability of health care or clinical services; and

(3) Any additional information required by the Authority for the purposes specified in this paragraph;

(c) Require the grantee to submit to the Authority an annual report that includes, without limitation, details regarding the financial status of the project, efforts to recruit providers of health care and the impact of the project;

(d) Require the grantee to comply with any request made by the Authority or the Office of Finance as part of a site visit, audit or review conducted pursuant to section 16 of this act;

(e) Specify the date on which the portion of the project funded by the grant is scheduled to terminate; and

(f) Require the grantee to meet any additional conditions imposed by the Authority to ensure accountability, the achievement of the objectives of the project and the sustainability of the project.

3. A funding agreement entered into pursuant to subsection 1 may require that any matching funds be:

(a) Held in escrow by an independent third party before the disbursement of money from the Account;

(b) Secured through an irrevocable trust, letter of credit or surety bond; or

(c) Verified through financial reporting to confirm the availability and appropriate use of the matching funds.

4. A funding agreement entered into pursuant to subsection 1 may require that the matching funds and in-kind contributions, if required, be subject to an independent verification process that may include, without limitation:

(a) Certification by a certified public accountant or other qualified third party;

(b) Submission of documentation demonstrating the value and availability of matching funds or in-kind contributions, including, without limitation:

(1) Financial statements;



- (2) Governance documents;
- (3) Appraisals or market valuations of in-kind contributions; and
- (4) Documentation of cost basis or fair market value, as appropriate;
- (c) Periodic reports concerning the valuation of in-kind contributions that are aligned with the disbursement schedule established pursuant to paragraph (a) of subsection 2; and
- (d) Adherence to generally accepted accounting principles for the valuation of in-kind contributions.

Sec. 15. 1. *If a grantee significantly modifies a project that has received funding pursuant to sections 2 to 18, inclusive, of this act, or if there are changes in the availability of funding for such a project, the grantee shall immediately notify the Authority and submit to the Authority a revised plan for the project. Upon receiving such notification, the Authority may, to ensure that money from the Account is used efficiently and effectively in a manner that complies with section 10 of this act, adjust the terms of the funding agreement, including, without limitation, the terms for disbursement and the amount of funding, or suspend or terminate the funding agreement.*

2. *If a grantee terminates a project that has received funding pursuant to sections 2 to 18, inclusive, of this act, before the date of termination specified in the applicable funding agreement or if such a project remains inactive for 12 consecutive months, the grantee shall:*

- (a) *Notify the Authority; and*
 - (b) *Submit to the Authority a revised plan for the project.*
 - 3.** *Upon receiving notice pursuant to subsection 2 of the early termination or inactivity of a project, the Authority may:*
 - (a) *Place a lien or other security interest on assets related to the project until at least half of the goals and indicators of performance included in the applicable funding agreement are met;*
 - (b) *Reallocate unspent money to other approved projects or deposit unspent money in the Account to support grants for other projects pursuant to sections 2 to 18, inclusive, of this act;*
 - (c) *Require the grantee to take corrective action within 60 days;*
 - (d) *Suspend or terminate the grant;*
 - (e) *Require the grantee to repay money previously disbursed;*
- or*



(f) Take any combination of the actions described in paragraphs (a) to (e), inclusive.

4. Not later than 60 days after the early termination of a project for which a grantee received a grant pursuant to sections 2 to 18, inclusive, of this act, a grantee shall submit to the Authority a report which includes, without limitation:

(a) The total amount of money spent on the project;

(b) The amount of money disbursed from the Account that was spent on the project;

(c) The reasons for the early termination of the project; and

(d) A proposed plan to repay money disbursed from the Account, if required by the Authority.

Sec. 16. 1. The Authority shall conduct site visits, audits or reviews at least annually to ensure that a project funded pursuant to sections 2 to 18, inclusive, of this act complies with the requirements of those sections and the applicable funding agreement.

2. The Office of Finance may audit any project funded pursuant to sections 2 to 18, inclusive, of this act to ensure that the grantee is using the money awarded pursuant to sections 2 to 18, inclusive, of this act in an effective and efficient manner that accords with state and federal law and the applicable funding agreement.

Sec. 17. If the Authority concludes, as the result of a site visit, audit or review conducted pursuant to section 16 of this act or for any other reason, that a grantee who has received funding pursuant to sections 2 to 18, inclusive, of this act has failed to:

1. Comply with the terms of a funding agreement, including, without limitation, by failing to achieve the milestones, goals and indicators of performance prescribed in the funding agreement pursuant to paragraph (a) of subsection 2 of section 14 of this act, the Authority may:

(a) Suspend or terminate the grant;

(b) Require the grantee to take corrective action within 60 days;

(c) Require the grantee to repay money that was previously disbursed to the grantee;

(d) Reallocate unspent money to other approved projects or deposit unspent money in the Account to support grants for other projects pursuant to sections 2 to 18, inclusive, of this act;

(e) Take such other measures as are necessary to ensure compliance with the provisions of sections 2 to 18, inclusive, of



this act, the regulations adopted pursuant thereto, other state and federal law and the applicable funding agreement; or

(f) Take any combination of the actions described in paragraphs (a) to (e), inclusive.

2. Comply with any provision of sections 2 to 18, inclusive, of this act, any regulation adopted pursuant thereto or any other state or federal law, or has misused funds or submitted fraudulent information to the Authority, the Authority:

(a) Shall suspend or terminate the grant;

(b) May refer the matter to a district attorney who has jurisdiction over the matter or the Attorney General, where appropriate; and

(c) May additionally take any other action or combination of actions described in subsection 1.

Sec. 18. *Upon completing a project for which a grantee received a grant pursuant to sections 2 to 18, inclusive, of this act, and again 10 years after the completion of such a project, the grantee shall submit to the Authority a report that includes, without limitation:*

1. A summary of the achievements of the project relative to the initial goals of the project, focusing on improvements in access to health care or clinical services and the capacity of the health care workforce in the area affected by the project;

2. Financial accounting of all money received for the project pursuant to sections 2 to 18, inclusive, of this act and from other sources, including, without limitation, any matching funds, and an itemized statement of expenditures of such money;

3. A narrative evaluation of the impact of the project on the community affected by the project, including, without limitation, benefits realized, challenges encountered and lessons learned for future projects;

4. Any relevant data on patient outcomes, measurements of community health or the retention and expansion of the health care workforce attributable to the project; and

5. Feedback or testimonials concerning the project from beneficiaries, partners and other affected persons and entities.

Sec. 19. Chapter 449 of NRS is hereby amended by adding thereto a new section to read as follows:

1. A hospital shall, for at least 95 percent of the complete requests for privileging submitted by providers of health care to the hospital, process the request not later than 60 days after the hospital receives all information necessary to complete the request.



2. Not later than 15 days after a hospital receives an incomplete request for privileging from a provider of health care, the hospital shall notify the provider of health care of the information necessary to complete the request.

3. On or before December 1 of each year, a hospital shall compile and submit to the Division a report on the privileging of providers of health care which includes, without limitation:

(a) The average time between the submission by a provider of health care of a request for privileging and the request being approved or denied;

(b) The rates at which the hospital processes requests for privileging within the time period specified in subsection 1; and

(c) Any planned improvements to the hospital's process for privileging providers of health care, including, without limitation, improvements to technology or procedures to increase the efficiency of the process.

4. A hospital that fails to comply with the provisions of this section is not subject to any penalty imposed pursuant to this chapter for such failure to comply.

5. As used in this section, "privileging" means the process of determining whether to authorize a provider of health care to provide specific services at a hospital based on his or her credentials and qualifications.

Sec. 20. NRS 449.029 is hereby amended to read as follows:

449.029 As used in NRS 449.029 to 449.240, inclusive, **and section 19 of this act**, unless the context otherwise requires, "medical facility" has the meaning ascribed to it in NRS 449.0151 and includes a program of hospice care described in NRS 449.196.

Sec. 21. NRS 449.0301 is hereby amended to read as follows:

449.0301 The provisions of NRS 449.029 to 449.2428, inclusive, **and section 19 of this act** do not apply to:

1. Any facility conducted by and for the adherents of any church or religious denomination for the purpose of providing facilities for the care and treatment of the sick who depend solely upon spiritual means through prayer for healing in the practice of the religion of the church or denomination, except that such a facility shall comply with all regulations relative to sanitation and safety applicable to other facilities of a similar category.

2. Foster homes as defined in NRS 424.014.

3. Any medical facility, facility for the dependent or facility which is otherwise required by the regulations adopted by the Board pursuant to NRS 449.0303 to be licensed that is operated and maintained by the United States Government or an agency thereof.



Sec. 22. NRS 449.160 is hereby amended to read as follows:

449.160 1. Except as otherwise provided in NRS 449.1887 ~~§~~ **and section 19 of this act**, the Division may deny an application for a license or registration or may suspend or revoke any license or registration issued under the provisions of NRS 449.029 to 449.2428, inclusive, **and section 19 of this act** upon any of the following grounds:

(a) Violation by the applicant, registrant or licensee of any of the provisions of NRS 439B.410, 442.010, 449.029 to 449.245, inclusive, **and section 19 of this act** or NRS 449A.100 to 449A.124, inclusive, and 449A.270 to 449A.286, inclusive, or of any other law of this State or of the standards, rules and regulations adopted thereunder.

(b) Aiding, abetting or permitting the commission of any illegal act.

(c) Conduct inimical to the public health, morals, welfare and safety of the people of the State of Nevada in the maintenance and operation of the premises for which a license is issued.

(d) Conduct or practice detrimental to the health or safety of the occupants or employees of the facility.

(e) Failure of the applicant to obtain written approval from the Director of the Department of Human Services as required by NRS 439A.100 or 439A.102 or as provided in any regulation adopted pursuant to NRS 449.001 to 449.430, inclusive, **and section 19 of this act** and 449.435 to 449.531, inclusive, and chapter 449A of NRS if such approval is required, including, without limitation, the closure or conversion of any hospital in a county whose population is 100,000 or more that is owned by the licensee without approval pursuant to NRS 439A.102.

(f) Failure to comply with the provisions of NRS 441A.315 and any regulations adopted pursuant thereto or NRS 449.2486.

(g) Violation of the provisions of NRS 458.112.

(h) Failure to comply with the provisions of NRS 449A.170 to 449A.192, inclusive, and any regulation adopted pursuant thereto.

(i) Violation of the provisions of NRS 629.260.

2. In addition to the provisions of subsection 1, the Division may revoke a license to operate a facility for the dependent if, with respect to that facility, the licensee that operates the facility, or an agent or employee of the licensee:

(a) Is convicted of violating any of the provisions of NRS 202.470;

(b) Is ordered to but fails to abate a nuisance pursuant to NRS 244.360, 244.3603 or 268.4124; or



(c) Is ordered by the appropriate governmental agency to correct a violation of a building, safety or health code or regulation but fails to correct the violation.

3. The Division shall maintain a log of any complaints that it receives relating to activities for which the Division may revoke the license to operate a facility for the dependent pursuant to subsection 2. The Division shall provide to a facility for the care of adults during the day:

(a) A summary of a complaint against the facility if the investigation of the complaint by the Division either substantiates the complaint or is inconclusive;

(b) A report of any investigation conducted with respect to the complaint; and

(c) A report of any disciplinary action taken against the facility.
➔ The facility shall make the information available to the public pursuant to NRS 449.2486.

4. On or before February 1 of each odd-numbered year, the Division shall submit to the Director of the Legislative Counsel Bureau a written report setting forth, for the previous biennium:

(a) Any complaints included in the log maintained by the Division pursuant to subsection 3; and

(b) Any disciplinary actions taken by the Division pursuant to subsection 2.

Sec. 23. NRS 449.163 is hereby amended to read as follows:

449.163 1. Except as otherwise provided in NRS 449.1887 **and section 19 of this act**, in addition to the payment of the amount required by NRS 449.0308, if a medical facility, facility for the dependent or facility which is required by the regulations adopted by the Board pursuant to NRS 449.0303 to be licensed violates any provision related to its licensure, including any provision of NRS 439B.410, 442.010 or 449.029 to 449.2428, inclusive, **and section 19 of this act** or any condition, standard or regulation adopted by the Board, the Division, in accordance with the regulations adopted pursuant to NRS 449.165, may:

(a) Prohibit the facility from admitting any patient until it determines that the facility has corrected the violation;

(b) Limit the occupancy of the facility to the number of beds occupied when the violation occurred, until it determines that the facility has corrected the violation;

(c) If the license of the facility limits the occupancy of the facility and the facility has exceeded the approved occupancy, require the facility, at its own expense, to move patients to another facility that is licensed;



(d) Except where a greater penalty is authorized by subsection 2, impose an administrative penalty of not more than \$5,000 per day for each violation, together with interest thereon at a rate not to exceed 10 percent per annum; and

(e) Appoint temporary management to oversee the operation of the facility and to ensure the health and safety of the patients of the facility, until:

(1) It determines that the facility has corrected the violation and has management which is capable of ensuring continued compliance with the applicable statutes, conditions, standards and regulations; or

(2) Improvements are made to correct the violation.

2. If an off-campus location of a hospital fails to obtain a national provider identifier that is distinct from the national provider identifier used by the main campus and any other off-campus location of the hospital in violation of NRS 449.1818, the Division may impose against the hospital an administrative penalty of not more than \$10,000 for each day of such failure, together with interest thereon at a rate not to exceed 10 percent per annum, in addition to any other action authorized by this chapter.

3. If the facility fails to pay any administrative penalty imposed pursuant to paragraph (d) of subsection 1 or subsection 2, the Division may:

(a) Suspend the license of the facility until the administrative penalty is paid; and

(b) Collect court costs, reasonable attorney's fees and other costs incurred to collect the administrative penalty.

4. Except as otherwise provided in NRS 449.1887  **and section 19 of this act**, the Division may require any facility that violates any provision of NRS 439B.410 or 449.029 to 449.2428, inclusive, **and section 19 of this act** or any condition, standard or regulation adopted by the Board to make any improvements necessary to correct the violation.

5. Any money collected as administrative penalties pursuant to paragraph (d) of subsection 1 or subsection 2 must be accounted for separately and used to administer and carry out the provisions of NRS 449.001 to 449.430, inclusive, **and section 19 of this act**, 449.435 to 449.531, inclusive, and chapter 449A of NRS to protect the health, safety, well-being and property of the patients and residents of facilities in accordance with applicable state and federal standards or for any other purpose authorized by the Legislature.



Sec. 24. NRS 449.240 is hereby amended to read as follows:

449.240 Except as otherwise provided in NRS 449.1887 ~~§~~ *and section 19 of this act*, the district attorney of the county in which the facility is located shall, upon application by the Division, institute and conduct the prosecution of any action for violation of any provisions of NRS 449.029 to 449.245, inclusive ~~§~~ , *and section 19 of this act*.

Sec. 24.5. NRS 454.213 is hereby amended to read as follows:

454.213 1. Except as otherwise provided in NRS 454.217, a drug or medicine referred to in NRS 454.181 to 454.371, inclusive, may be possessed and administered by:

(a) A practitioner.

(b) A physician assistant licensed pursuant to chapter 630 or 633 of NRS or an anesthesiologist assistant, at the direction of his or her supervising physician or supervising anesthesiologist or supervising osteopathic anesthesiologist, as applicable, or a licensed dental hygienist or expanded function dental assistant acting in the office of and under the supervision of a dentist.

(c) Except as otherwise provided in paragraph (d), a registered nurse licensed to practice professional nursing or licensed practical nurse, at the direction of a prescribing physician, physician assistant licensed pursuant to chapter 630 or 633 of NRS, dentist, podiatric physician or advanced practice registered nurse, or pursuant to a chart order, for administration to a patient at another location.

(d) In accordance with applicable regulations of the Board, a registered nurse licensed to practice professional nursing or licensed practical nurse who is:

(1) Employed by a health care agency or health care facility that is authorized to provide emergency care, or to respond to the immediate needs of a patient, in the residence of the patient; and

(2) Acting under the direction of the medical director of that agency or facility who works in this State.

(e) A medication aide - certified at a designated facility under the supervision of an advanced practice registered nurse or registered nurse and in accordance with standard protocols developed by the State Board of Nursing. As used in this paragraph, “designated facility” has the meaning ascribed to it in NRS 632.0145.

(f) Except as otherwise provided in paragraph (g), an advanced emergency medical technician or a paramedic, as authorized by regulation of the State Board of Pharmacy and in accordance with any applicable regulations of:



(1) The State Board of Health in a county whose population is less than 100,000;

(2) A county board of health in a county whose population is 100,000 or more; or

(3) A district board of health created pursuant to NRS 439.362 or 439.370 in any county.

(g) An advanced emergency medical technician or a paramedic who holds an endorsement issued pursuant to NRS 450B.1975, under the direct supervision of a local health officer or a designee of the local health officer pursuant to that section.

(h) A respiratory therapist employed in a health care facility. The therapist may possess and administer respiratory products only at the direction of a physician.

(i) A dialysis technician, under the direction or supervision of a physician or registered nurse only if the drug or medicine is used for the process of renal dialysis.

(j) A medical student or student nurse in the course of his or her studies at an accredited college of medicine or approved school of professional or practical nursing, at the direction of a physician and:

(1) In the presence of a physician or a registered nurse; or

(2) Under the supervision of a physician or a registered nurse if the student is authorized by the college or school to administer the drug or medicine outside the presence of a physician or nurse.

➤ A medical student or student nurse may administer a dangerous drug in the presence or under the supervision of a registered nurse alone only if the circumstances are such that the registered nurse would be authorized to administer it personally.

(k) Any person designated by the head of a correctional institution.

(l) An ultimate user or any person designated by the ultimate user pursuant to a written agreement.

(m) A holder of a license to engage in radiation therapy and radiologic imaging issued pursuant to chapter 653 of NRS, at the direction of a physician and in accordance with any conditions established by regulation of the Board.

(n) A chiropractic physician, but only if the drug or medicine is a topical drug used for cooling and stretching external tissue during therapeutic treatments.

(o) A physical therapist, but only if the drug or medicine is a topical drug which is:

(1) Used for cooling and stretching external tissue during therapeutic treatments; and

(2) Prescribed by a licensed physician for:



(I) Iontophoresis; or
(II) The transmission of drugs through the skin using ultrasound.

(p) In accordance with applicable regulations of the State Board of Health, an employee of a residential facility for groups, as defined in NRS 449.017, pursuant to a written agreement entered into by the ultimate user.

(q) A veterinary technician or a veterinary assistant at the direction of his or her supervising veterinarian.

(r) In accordance with applicable regulations of the Board, a registered pharmacist who:

(1) Is trained in and certified to carry out standards and practices for immunization programs;

(2) Is authorized to administer immunizations pursuant to written protocols from a physician; and

(3) Administers immunizations in compliance with ~~the~~ :

(I) Except where modified by the State Board of Health pursuant to sub-subparagraph (III), the version of “Standards for Immunization Practices” recommended ~~and approved~~ by the National Vaccine Advisory Committee of the United States Department of Health and Human Services that was in effect on January 1, 2025;

(II) Except where modified by the State Board of Health pursuant to sub-subparagraph (III), the recommendations of the Advisory Committee on Immunization Practices of the Centers for Disease Control and Prevention ~~is~~ in effect on January 1, 2025; and

(III) Any modifications to the standards and recommendations listed in sub-subparagraphs (I) and (II) or any additional standards or recommendations as the State Board of Health may prescribe pursuant to subsection 2.

(s) A registered pharmacist pursuant to written guidelines and protocols developed pursuant to NRS 639.2629 or a collaborative practice agreement, as defined in NRS 639.0052.

(t) A person who is enrolled in a training program to become a physician assistant or anesthesiologist assistant licensed pursuant to chapter 630 or 633 of NRS, dental hygienist, advanced emergency medical technician, paramedic, respiratory therapist, dialysis technician, physical therapist or veterinary technician or to obtain a license to engage in radiation therapy and radiologic imaging pursuant to chapter 653 of NRS if the person possesses and administers the drug or medicine in the same manner and under the same conditions that apply, respectively, to a physician assistant or



anesthesiologist assistant licensed pursuant to chapter 630 or 633 of NRS, dental hygienist, advanced emergency medical technician, paramedic, respiratory therapist, dialysis technician, physical therapist, veterinary technician or person licensed to engage in radiation therapy and radiologic imaging who may possess and administer the drug or medicine, and under the direct supervision of a person licensed or registered to perform the respective medical art or a supervisor of such a person.

(u) A medical assistant, in accordance with applicable regulations of the:

(1) Board of Medical Examiners, at the direction of the prescribing physician and under the supervision of a physician or physician assistant.

(2) State Board of Osteopathic Medicine, at the direction of the prescribing physician and under the supervision of a physician or physician assistant.

2. *The State Board of Health may, by regulation, modify any standard or recommendation described in subparagraph (3) of paragraph (r) of subsection 1 or adopt such additional standards and recommendations for the administration of immunizations by registered pharmacists as the Board deems necessary.*

3. As used in this section, “accredited college of medicine” has the meaning ascribed to it in NRS 453.375.

Sec. 25. NRS 218E.405 is hereby amended to read as follows:

218E.405 1. Except as otherwise provided in subsection 2, the Interim Finance Committee may exercise the powers conferred upon it by law only when the Legislature is not in a regular or special session.

2. During a regular or special session, the Interim Finance Committee may also perform the duties imposed on it by NRS 228.1111, 232.49943, subsection 5 of NRS 284.115, NRS 285.070, subsection 2 of NRS 321.335, NRS 322.007, subsection 2 of NRS 323.020, NRS 323.050, subsection 1 of NRS 323.100, subsection 3 of NRS 341.126, NRS 341.142, paragraph (f) of subsection 1 of NRS 341.145, subsection 3 of NRS 349.073, NRS 353.220, 353.224, 353.2705 to 353.2771, inclusive, 353.288, 353.335, 353.3375, 353C.224, 353C.226, paragraph (b) of subsection 6 of NRS 407.0762, NRS 428.375, 433.732, 439.4905, 439.620, 439.630, 445B.830, subsection 1 of NRS 445C.320, NRS 538.650 and 580.250 *and sections 8 and 13 of this act.* In performing those duties, the Senate Standing Committee on Finance and the Assembly Standing Committee on Ways and Means may meet separately and transmit the results of their respective votes to the



Chair of the Interim Finance Committee to determine the action of the Interim Finance Committee as a whole.

3. The Chair of the Interim Finance Committee may appoint a subcommittee consisting of six members of the Committee to review and make recommendations to the Committee on matters of the State Public Works Division of the Department of Administration that require prior approval of the Interim Finance Committee pursuant to subsection 3 of NRS 341.126, NRS 341.142 and paragraph (f) of subsection 1 of NRS 341.145. If the Chair appoints such a subcommittee:

(a) The Chair shall designate one of the members of the subcommittee to serve as the chair of the subcommittee;

(b) The subcommittee shall meet throughout the year at the times and places specified by the call of the chair of the subcommittee; and

(c) The Director or the Director's designee shall act as the nonvoting recording secretary of the subcommittee.

Sec. 26. NRS 232.4993 is hereby amended to read as follows:

232.4993 1. The Authority shall administer:

(a) The provisions of chapters 446, 449, 449A, 634B, 640D, 640E, 652, 695I and 695K of NRS and NRS 232.4996 to 232.49969, inclusive, 287.0402 to 287.049, inclusive, and 422.001 to 422.410, inclusive, and 422.580, 439.258, 439.271 to 439.2794, inclusive, 439.581 to 439.597, inclusive, 439.800 to 439.918, inclusive, 439A.200 to 439A.290, inclusive, 439B.600 to 439B.695, inclusive, 439B.800 to 439B.875, inclusive, and 444.003 to 444.430, inclusive, *and sections 2 to 18, inclusive, of this act*, and all other provisions of law relating to the functions of the divisions of the Authority; and

(b) Any state program for persons with developmental disabilities established pursuant to the Developmental Disabilities Assistance and Bill of Rights Act of 2000, 42 U.S.C. §§ 15001 et seq.

2. The Governor may delegate functions to the Authority other than those described in NRS 232.49903 to 232.49945, inclusive.

Sec. 27. NRS 233B.038 is hereby amended to read as follows:

233B.038 1. "Regulation" means:

(a) An agency rule, standard, directive or statement of general applicability which effectuates or interprets law or policy, or describes the organization, procedure or practice requirements of any agency;

(b) A proposed regulation;

(c) The amendment or repeal of a prior regulation; and



(d) The general application by an agency of a written policy, interpretation, process or procedure to determine whether a person is in compliance with a federal or state statute or regulation in order to assess a fine, monetary penalty or monetary interest.

2. The term does not include:

(a) A statement concerning only the internal management of an agency and not affecting private rights or procedures available to the public;

(b) A declaratory ruling;

(c) An intraagency memorandum;

(d) A manual of internal policies and procedures or audit procedures of an agency which is used solely to train or provide guidance to employees of the agency and which is not used as authority in a contested case to determine whether a person is in compliance with a federal or state statute or regulation;

(e) An agency decision or finding in a contested case;

(f) An advisory opinion issued by an agency that is not of general applicability;

(g) A published opinion of the Attorney General;

(h) An interpretation of an agency that has statutory authority to issue interpretations;

(i) Letters of approval, concurrence or disapproval issued in relation to a permit for a specific project or activity;

(j) A contract or agreement into which an agency has entered;

(k) The provisions of a federal law, regulation or guideline;

(l) An emergency action taken by an agency that is necessary to protect public health and safety;

(m) The application by an agency of a policy, interpretation, process or procedure to a person who has sufficient prior actual notice of the policy, interpretation, process or procedure to determine whether the person is in compliance with a federal or state statute or regulation in order to assess a fine, monetary penalty or monetary interest;

(n) A regulation concerning the use of public roads or facilities which is indicated to the public by means of signs, signals and other traffic-control devices that conform with the manual and specifications for a uniform system of official traffic-control devices adopted pursuant to NRS 484A.430;

(o) The classification of wildlife or the designation of seasons for hunting, fishing or trapping by regulation of the Board of Wildlife Commissioners pursuant to the provisions of title 45 of NRS; ~~for~~



(p) A technical bulletin prepared pursuant to NRS 360.133 **H** ;
or

(q) The assessment conducted by the Nevada Health Authority pursuant to section 9 of this act.

Sec. 28. Chapter 630 of NRS is hereby amended by adding thereto the provisions set forth as sections 29 and 30 of this act.

Sec. 29. 1. The Board shall adopt regulations establishing a procedure to prioritize the processing of applications for the initial issuance of a license to practice medicine submitted by an applicant who intends to practice:

(a) Serving geographic areas and populations of this State where the shortage of providers of health care is most critical, as identified by the assessment conducted pursuant to section 9 of this act; or

(b) In a specialty for which the need is most critical, as identified by the assessment conducted pursuant to section 9 of this act.

2. In establishing procedures to carry out the regulations adopted pursuant to this section, the Board shall:

(a) Establish metrics to monitor the processing times of applications described in subsection 1 to ensure compliance with the requirements of that subsection; and

(b) In collaboration with the Nevada Health Authority, annually update the geographic areas, populations and specialties for which applications are prioritized in response to each assessment conducted pursuant to section 9 of this act.

Sec. 30. 1. The Board shall:

(a) Establish an electronic system to allow an entity that verifies the credentials of providers of health care or a hospital to access data in the possession of the Board for the purpose of privileging or credentialing a physician, physician assistant, anesthesiologist assistant, genetic counselor, perfusionist or practitioner of respiratory care who has authorized the Board to share such data pursuant to paragraph (b).

(b) Allow an applicant for the issuance of a license to practice medicine, a physician applying for biennial registration or an applicant for the issuance or renewal of a license as a physician assistant, anesthesiologist assistant, genetic counselor, perfusionist or practitioner of respiratory care to indicate whether he or she wishes to allow electronic access to his or her data pursuant to paragraph (a).

2. As used in this section:



(a) “Credentialing” means verifying the credentials of a provider of health care for the purpose of determining whether the provider of health care meets the requirements for participation in the network of a third party or participation in Medicaid or the Children’s Health Insurance Program as a provider of services.

(b) “Network” has the meaning ascribed to it in NRS 687B.640.

Sec. 31. NRS 630.130 is hereby amended to read as follows:

630.130 1. In addition to the other powers and duties provided in this chapter, the Board shall, in the interest of the public, judiciously:

(a) Enforce the provisions of this chapter;

(b) Establish by regulation standards for licensure under this chapter;

(c) Investigate the character of each applicant for a license and issue licenses to those applicants who meet the qualifications set by this chapter and the Board; and

(d) Institute a proceeding in any court to enforce its orders or the provisions of this chapter.

2. On or before February 15 of each odd-numbered year, the Board shall submit to the Governor and to the Director of the Legislative Counsel Bureau for transmittal to the next regular session of the Legislature a written report compiling:

(a) Disciplinary action taken by the Board during the previous biennium against any licensee for malpractice or negligence;

(b) Information reported to the Board during the previous biennium pursuant to NRS 630.3067, 630.3068, subsections 3 and 6 of NRS 630.307 and NRS 690B.250; ~~and~~

(c) Information reported to the Board during the previous biennium pursuant to NRS 630.30665, including, without limitation, the number and types of surgeries performed by each holder of a license to practice medicine and the occurrence of sentinel events arising from such surgeries, if any ~~H~~; and

(d) Information relating to the efficiency of the process for licensing physicians, including, without limitation:

(1) The average time during the immediately preceding biennium between when a person applied for a license to practice medicine and when the license was issued or the application was denied;

(2) The total number of applications for licensure to practice medicine processed by the Board during the immediately preceding biennium; and



(3) Recommendations for improvements to the process for licensing physicians.

➔ The report must include only aggregate information for statistical purposes and exclude any identifying information related to a particular person.

3. The Board may adopt such regulations as are necessary or desirable to enable it to carry out the provisions of this chapter. Such regulations may include, without limitation, regulations requiring, as a condition for licensure, that an applicant pass one or more examinations in addition to those required by this chapter and regulations concerning the scoring of any such examination.

Sec. 32. Chapter 633 of NRS is hereby amended by adding thereto the provisions set forth as sections 33 and 34 of this act.

Sec. 33. 1. The Board shall adopt regulations establishing a procedure to prioritize the processing of applications for the initial issuance of a license to practice osteopathic medicine submitted by an applicant who intends to practice:

(a) Serving geographic areas and populations of this State where the shortage of providers of health care is most critical, as identified by the assessment conducted pursuant to section 9 of this act; or

(b) In a specialty for which the need is most critical, as identified by the assessment conducted pursuant to section 9 of this act.

2. In establishing procedures to carry out the regulations adopted pursuant to this section, the Board shall:

(a) Establish metrics to monitor the processing times of applications described in subsection 1 to ensure compliance with the requirements of that subsection; and

(b) In collaboration with the Nevada Health Authority, annually update the geographic areas, populations and specialties for which applications are prioritized in response to each assessment conducted pursuant to section 9 of this act.

Sec. 34. 1. The Board shall:

(a) Establish an electronic system to allow an entity that verifies the credentials of providers of health care or a hospital to access data in the possession of the Board for the purpose of privileging or credentialing an osteopathic physician, physician assistant or anesthesiologist assistant who has authorized the Board to share such data pursuant to paragraph (b).

(b) Allow an applicant for the issuance or renewal of a license as an osteopathic physician, physician assistant or anesthesiologist



assistant to indicate whether he or she wishes to allow electronic access to his or her data pursuant to paragraph (a).

2. As used in this section:

(a) “Credentialing” means verifying the credentials of a provider of health care for the purpose of determining whether the provider of health care meets the requirements for participation in the network of a third party or participation in Medicaid or the Children’s Health Insurance Program as a provider of services.

(b) “Network” has the meaning ascribed to it in NRS 687B.640.

(c) “Privileging” means the process of determining whether to authorize a provider of health care to provide specific services at a hospital based on his or her credentials and qualifications.

(d) “Third party” means any insurer, governmental entity or other organization providing health coverage or benefits in accordance with state or federal law.

Sec. 35. NRS 633.286 is hereby amended to read as follows:

633.286 1. On or before February 15 of each odd-numbered year, the Board shall submit to the Governor and to the Director of the Legislative Counsel Bureau for transmittal to the next regular session of the Legislature a written report compiling:

(a) Disciplinary action taken by the Board during the previous biennium against osteopathic physicians, physician assistants and anesthesiologist assistants for malpractice or negligence;

(b) Information reported to the Board during the previous biennium pursuant to NRS 633.526, 633.527, subsections 3 and 6 of NRS 633.533 and NRS 690B.250; ~~and~~

(c) Information reported to the Board during the previous biennium pursuant to NRS 633.524, including, without limitation, the number and types of surgeries performed by each holder of a license to practice osteopathic medicine and the occurrence of sentinel events arising from such surgeries, if any ~~and~~; *and*

(d) Information relating to the efficiency of the process for licensing osteopathic physicians, including, without limitation:

(1) The average time during the immediately preceding biennium between when a person applied for a license to practice osteopathic medicine and when the license was issued or the application was denied;

(2) The total number of applications for licensure to practice osteopathic medicine processed by the Board during the immediately preceding biennium; and

(3) Recommendations for improvements to the process for licensing osteopathic physicians.



2. The report must include only aggregate information for statistical purposes and exclude any identifying information related to a particular person.

Sec. 36. There is hereby appropriated from the State General Fund to the Statewide Health Care Access and Recruitment Program Account created in the State General Fund by section 8 of this act the sum of \$60,000,000 for the purposes described in subsections 3 and 4 of section 8 of this act.

Sec. 37. 1. There is hereby appropriated from the State General Fund to the Board of Medical Examiners for the purpose of carrying out the provisions of sections 29 and 30 of this act and paragraph (d) of subsection 2 of NRS 630.130, as amended by section 31 of this act, the following sums:

For the Fiscal Year 2025-2026..... \$250,000

For the Fiscal Year 2026-2027..... \$250,000

2. There is hereby appropriated from the State General Fund to the State Board of Osteopathic Medicine for the purpose of carrying out the provisions of sections 33 and 34 of this act and paragraph (d) of subsection 1 of NRS 633.286, as amended by section 35 of this act, the following sums:

For the Fiscal Year 2025-2026..... \$250,000

For the Fiscal Year 2026-2027..... \$250,000

3. Any balance of the sums appropriated by subsections 1 and 2 remaining at the end of the respective fiscal years must not be committed for expenditure after June 30 of the respective fiscal years by the entity to which the appropriation is made or any entity to which money from the appropriation is granted or otherwise transferred in any manner, and any portion of the appropriated money remaining must not be spent for any purpose after September 18, 2026, and September 17, 2027, respectively, by either the entity to which the money was appropriated or the entity to which the money was subsequently granted or transferred, and must be reverted to the State General Fund on or before September 18, 2026, and September 17, 2027, respectively.

Sec. 38. In lieu of conducting the assessment described in paragraph (a) of subsection 1 of section 9 of this act on or before July 1, 2026, the Nevada Health Authority shall:

1. Review existing assessments of the health care needs of this State that, to the extent available, carry out the purposes described in subsection 2 of section 9 of this act; and

2. Utilize the assessments described in subsection 1 to:

(a) Create the summary described in paragraph (a) of subsection 3 of section 9 of this act; and



(b) Make the recommendations described in paragraphs (b) and (c) of subsection 3 of section 9 of this act.

Sec. 39. The provisions of subsection 1 of NRS 218D.380 do not apply to any provision of this act which adds or revises a requirement to submit a report to the Legislature.

Sec. 40. The provisions of NRS 354.599 do not apply to any additional expenses of a local government that are related to the provisions of this act.

Sec. 41. Notwithstanding the provisions of NRS 218D.430, a committee may vote on this act before the period prescribed for the return of a fiscal note in NRS 218D.475.

Sec. 42. 1. This section, sections 1 to 18, inclusive, 24.5 to 27, inclusive, and 37 to 41, inclusive, of this act become effective upon passage and approval.

2. Sections 19 to 24, inclusive, and 28 to 36, inclusive, of this act become effective on July 1, 2026.

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